

14 May 2021

Ms Treasure Jennings, Commissioner
Mental Health Complaints Commission
Level 26, 570 Bourke Street
Melbourne VIC 3000

Attn: Ms Cathie Seccombe, Sessional Adviser Investigations and Review

Dear Ms Jennings,

Re: Complaint by Dr Richard (Rich) McLean - Reference 2021/00369

Thank you for your correspondence from Ms Kaaren Dahl, Acting Deputy Commissioner, dated 1 April 2021, outlining concerns in relation to the service provided to Mr Richard (Rich) McLean by Mercy Mental Health (MMH) Crisis Assessment and Treatment Team (CATT), Werribee Mercy Hospital Adult Mental Health Inpatient Unit (IPU) and Saltwater Community Mental Health Clinic, in February to March 2021. I would like to confirm that Dr McLean had contacted the Mercy Health Feedback Team in the first instance, expressing similar concerns and an internal investigation had already commenced.

In line with our feedback management process, the concerns have been investigated by the Deputy Clinical Services Director (Access & Residential), Deputy Clinical Services Director (Adult Inpatient), Deputy Clinical Services Director (Community). The investigation involved a review of Dr McLean's medical record, and discussions with the staff members involved in his care.

As requested by the Mental Health Complaints Commissioner (MHCC), responses to each of the concerns focus on information and actions pertaining to a resolution. Each point has been considered and responded to as set out below.

Crisis Assessment and Treatment Team (CATT)

Complaint Issue 1 – Staff at the CATT team visited Dr McLean's home in February 2021 and threatened him after he flippantly said he would kill himself.

- ***Dr McLean realised that the staff thought that he was serious, and he tried to convince them he was not suicidal***
- ***As the staff were on his property, Dr McLean recorded the visit and uploaded the recording to social media***
- ***The '4IC' nurse in the IPU later told him he had to remove it or face a \$40,000 fine***
- ***Dr McLean believes the relevant surveillance legislation allows him to record on his own property, and that the staff should have believed him when he advised that he was not suicidal***

MMH Response:

On 14 February 2021, Dr McLean was initially referred to MMH triage by a member of the public expressing concerns that his mental health may be declining as they had noticed that content on his website demonstrated an escalation in his distress, and had also listed suicide dates with increased frustration evident towards his former General Practitioner (GP) and health care professionals. It was also reported that Dr McLean has experienced increased stress due to recently losing his job as an art therapist with the National Disability Insurance Scheme (NDIS) and that his car had been repossessed.

On 15 February 2021, MMH triage received further concerns from Dr McLean's private psychiatrist who explained that he was no longer able to continue treatment as he had become increasingly hostile in his communication and had requested that his private psychiatrist fraudulently billed him in order for him to be charged less for his consultations. Dr McLean also requested another script for dexamphetamine as he reported that he had lost a recent script and medication supply. This could not be facilitated by his private psychiatrist as a new script was not due to be written and dexamphetamine is a highly monitored medication and practitioners require a permit to prescribe. The private psychiatrist also reported that Dr McLean had written them a letter expressing his frustration and suicidal ideation as a result of his perceived rejection of care. After MMH triage received this additional information Dr McLean was referred to MMH CATT for community follow up.

On 16 February 2021, Dr McLean was reviewed by the CATT consultant psychiatrist at Saltwater Clinic, Footscray. During this review, Dr McLean expressed frustration at being let down by the health system, particularly by his former GP who he reported did not respond to his risk of suicide some years ago, and has perceived this as mistreatment. During this review the above concerns for his mental health and wellbeing were discussed, and Dr McLean reported that publishing suicidal thoughts and intent was to draw attention to his current situation and that he was not actually suicidal. Dr McLean did not present as pervasively depressed, psychotic or displaying prominent Attention Deficit Hyperactivity Disorder (ADHD) symptoms during this review. However, a comprehensive assessment of ADHD was not performed at this time, as this is not a standard assessment completed at the point of crisis intervention.

It is noted that Dr McLean has a history of Schizophrenia and ADHD. The CATT consultant psychiatrist's overall impression after this assessment was that Dr McLean has a history of complex trauma, vulnerable personality traits of paranoid personality disorder spectrum and narcissistic personality disorder spectrum. It was also assessed that Dr McLean's risk of suicide was low and he denied any risks to others. It was recommended that Dr McLean seek a psychology referral to address his coping style and interpersonal difficulties.

During this review, Dr McLean was also advised that Area Mental Health Services, including MMH, do not prescribe dexamphetamine for ADHD and was provided with contact details for private psychiatrists who can offer this treatment. CATT contacted his GP to request an appropriate referral to a private psychiatrist to complete further assessment and prescriptions as required. CATT established that Dr McLean had a supply of dexamphetamine available, and had not run out of medication at this time. During this review, Dr McLean was provided with consent to record the interview, which was his preference, but he was informed that publishing of such recordings is illegal.

After this initial review, Dr McLean was accepted for CATT home treatment to provide short term support with the view that he would be referred back to his GP after a period of stability. However, after this initial review, MMH triage received further information that Dr McLean continued to post content on social media suggesting that he was suicidal.

On 23 February 2021, CATT attended Dr McLean's home, and after their initial assessment the clinicians felt that he required further assessment of his mental state and risks by a psychiatrist. Dr McLean was offered a voluntary admission which he declined. It was deemed that his clinical risks were significant enough, that community treatment was no longer an option, and his symptoms met the Mental Health Act 2014 Assessment Order criteria and was placed on an Inpatient Assessment Order and admitted.

Inpatient admission

Complaint Issue 2 – Dr McLean was admitted to the IPU on 23 February 2021 and his current and long-standing medication of 40mg dexamphetamine and 20mg paroxetine was ceased.

- ***Dr McLean believes that this was done to destabilise him, and could have resulted in serious withdrawal symptoms as he had been taking the medication for 3 years***
- ***Dr McLean was told that he was delusional***

MMH Response:

I would like to reassure Dr McLean that his medication was ceased based on evidence-based practice and clinical assessment and was not an attempt to destabilise him. As previously mentioned Dr McLean has a history of schizophrenia which he was being treated for, and his initial diagnosis at the time of his admission was acute psychosis.

Dexamphetamine is known to directly exacerbate psychotic symptoms and based on this evidence the clinical decision was made to immediately cease this medication. While we appreciate Dr McLean's concerns that sudden cessation of this medication may have resulted in withdrawal, it was considered that while withdrawal is a potential risk, this was likely to have less of an impact than if dexamphetamine was continued. It was also considered that the inpatient unit is well equipped to monitor and treat any potential withdrawal symptoms if this occurred. It is the informed opinion of the Deputy Clinical Services Director (Adult Inpatient) and the treating Consultant Psychiatrist that it is unlikely that the immediate cessation of dexamphetamine directly contributed to Dr McLean's attempted suicide. However, MMH will acknowledge that this additional stressor, coupled with pre-existing stressors and a deterioration in his mental health may have reduced his ability to manage suicidal thoughts or utilise alternative resources such as distraction, or seeking support from staff.

While it was not evident during his initial CATT assessment that Dr McLean was experiencing psychotic symptoms, it was later assessed that he was experiencing paranoid delusions, characterised by believing that recordings made without consent were admissible in court; despite being advised by Victoria Police and the Australian Health Practitioner Regulation Agency (AHPRA) that this is not admissible. Dr McLean continued to believe and express thoughts that there is a wide-ranging conspiracy against him, and also expressed concern that his treating Consultant Psychiatrist could also be part of the conspiracy against him. Dr McLean also demonstrated paranoid behaviour of wanting to record conversations with the treating team during his admission.

Complaint Issue 3 – The cessation of the medication led him to attempt suicide by cutting his arteries in the arm with broken glass from his vape pen, after he had attempted this with a broken toilet brush.

- ***The attempt resulted in him being admitted to ICU on 25 February 2021***
- ***The broken toilet brush was returned to him with his belongings, and he felt this was inappropriate***

MMH Response:

Please see information provided above in relation to Dr McLean's belief that the cessation of medication led to his suicide attempt.

I would like to apologise that Dr McLean felt that it was inappropriate for the broken toilet brush to be returned to him. It is not Mercy Health's process to give patients hospital property, and it was not until the IPU Nurse Unit Manager (NUM) spoke with Dr McLean's sister that we learned that this item was in her possession. At this time, it was unknown what role the toilet brush had played in the incident as he remained admitted to the Intensive Care Unit. The IPU NUM offered to remove and discard the toilet brush during their conversation, however Dr McLean had specifically requested his sister keep this as evidence as to what had happened. The IPU NUM was unsure if persisting in removing this item would cause further distress, and as an alternative provided his sister with education that returning this item to Dr McLean may in fact be triggering for him, and recommended that it was not returned to him or was well considered prior to returning it to him.

Complaint Issue 4 – The vape pen should have been categorised as contraband, and he says he brought it into the hospital accidentally.

MMH Response:

As previously confirmed during the initial discussion with the MHCC, the vape pen is classified as contraband and should not have been brought into the IPU. Through our investigation it was identified that the vape pen was removed from Dr McLean's possession on 24 February 2021, and at this time he was advised that this was a prohibited item establishing a mutual expectation.

Complaint Issue 5 – Dr McLean was only provided with his Temporary Treatment Order (TTO) on 1 March 2021, and was not provided with advocacy support or access to a lawyer, although he did speak with an IMHA advocate

- ***Dr McLean received his Statement of Rights 5 days into the admission***

MMH Response:

On 24 February 2021, Dr McLean was reviewed by a Consultant Psychiatrist and the clinical records demonstrate that during this meeting he was informed about his rights as a compulsory patient, how to appeal his compulsory treatment and that it is his right to access Legal Aid.

While Dr McLean received verbal education about his rights, there was a delay in providing him with a written copy. It was identified through our investigation that the following day Dr McLean was transferred to the Intensive Care Unit (ICU), and while the Consultation Liaison staff attempted to review Dr McLean in ICU, assessments were quickly terminated at the

request of Dr McLean by way of expletive language. This limited the ability for MMH to provide the written Statement of Rights.

Complaint Issue 6– Dr McLean’s mobile phone was removed, and he believes this was because he had previously recorded staff

MMH Response:

Dr McLean’s mobile phone was not removed in retaliation for recording staff, but because he was posting on social media, and it was necessary to protect his reputation. The removal was authorised by the treating Consultant Psychiatrist. The treating Consultant Psychiatrist established a plan that stipulated that Dr McLean’s phone use was to be supervised; however, optimising his access in a mutually agreed way to ensure he still had access to emails and could contact others, including family, friends, health professionals, advocacy services and legal services.

Complaint Issue 7 – When Dr McLean attempted suicide, he wrote a note to his family.

- ***Dr McLean’s sister asked for the note on his behalf, but was told it had been scanned into the clinical record and that they would need to make an FOI request to have the note returned.***
- ***The note is ‘sacred’ and important for Dr McLean and his family***

MMH Response:

MMH agree to return a scanned copy of the note to Dr McLean, and can facilitate this through his case manager at Saltwater Clinic.

Saltwater Clinic

Complaint Issue 8 – Dr McLean attended Saltwater Clinic on 15 March 2021 after he was referred by Dr Michael Lograsso, Deputy Clinical Services Director for ongoing psychosocial support.

- ***Dr McLean was not offered any support from a psychiatrist or social worker***

MMH Response:

Dr McLean was reviewed at Saltwater Clinic by a medical officer, during this assessment he was offered psychosocial supports such as psychology and/or social work input, which he declined. It appeared that Dr McLean’s purpose for attending Saltwater Clinic was to obtain a script for dexamphetamine, and when this was declined he left the clinic.

Dr McLean was further reviewed by the Consultant Psychiatrist on 24 March 2021.

Additional Information:

1. **Dr McLean completed a form to request a second psychiatric opinion early in his inpatient admission, but he did not hear from the service.**

MMH Response:

As the Victorian Second Psychiatric Opinion Service is an independent service, MMH are not in a position to provide comment.

- 2. Dr McLean is particularly upset by the refusal of service by staff at Saltwater Clinic after he had been dislocated from his home and job by being admitted as an inpatient, as he could not access the support he needed.**

MMH Response:

Please see response provided in complaint issue 8.

- 3. Dr McLean also did not understand why the clinic has a policy of not providing dexamphetamine when the abrupt withdrawal of this medication in the inpatient unit led to the suicide attempt
*Dr McLean has not had any follow up and is now unemployed and homeless and is 'squatting'***

MMH Response:

Please see response provided in complaint issue 1.

- 4. *When Dr McLean was admitted to the IPU, there was no one to care for his dog and the consultant said it would be in the pound in 2 days which was very distressing as he had never been away from his dog before.***

MMH Response:

I would like to apologise for the distress caused to Dr McLean by being separated from his dog. During his admission Dr McLean was advised that the social worker would be notified about the situation with his dog, so that appropriate arrangements could be made to have his dog cared for by the Royal Society for the Prevention of Cruelty to Animals (RSPCA). At no point did the treating team plan to have his dog taken to the pound.

- 5. Dr McLean was moved 5 times during his stay, and was not allowed to vape or smoke which caused further distress.**

MMH Response:

I offer my apologies if Dr McLean felt that his admission was disjointed; however, the moves between areas and/or units were required to ensure he received the most appropriate care for both his physical health and mental health. While I appreciate that not being allowed to smoke in certain areas of our hospital including the ICU and the Intensive Care Area (ICA) within the mental health IPU can be distressing, Mercy Health's first priority must be the health and safety of both patients and staff, while abiding by relevant legislation prohibiting smoking within four meters of an entrance to any public hospital. As an alternative, Dr McLean was offered nicotine replacement therapy during his admission.

- 6. Dr McLean believes he was within his rights to record the staff from the CATT team, as they were in his home, and for the protection of his life.**

MMH Response:

In Victoria, it is not illegal to secretly record or film another person in a private conversation or activity without their permission if the person making the recording is part of the conversation or activity. However, in Victoria, it is illegal to publish, send or distribute this material without the permission of each individual identified in the recording or images. In this instance, Dr McLean did not have consent from the CATT clinicians to publish this video.

- 7. Dr McLean only received a prescription for antipsychotic medication when he attended Saltwater Clinic**

MMH Response:

MMH provide patients with a 7-day supply of prescribed medication upon discharge from the inpatient unit, further prescriptions are provided through community follow up arrangements, and in Dr McLean's case this prescription was provided by Saltwater Clinic.

I trust that this letter addresses all of the concerns raised by the MHCC on behalf of Dr McLean. If you have any further questions or concerns, please do not hesitate to contact our Feedback Team on 8416 7783 or feedback@mercy.com.au.

Yours sincerely,



Jason Payne
Chief Executive Health Services
Mercy Health